

Documentation of Patient Care Report

Standard:

Establish the minimum documentation requirements for every patient contact.

Purpose:

To provide consistent and accurate documentation of DSHS reportable events following Tex. Admin Code Rule (TAC) [§ 103.4](#), the ATCEMS System Medical Director is responsible for designating the minimum data required for patient care reporting. The following are the minimum requirements for documentation on incidents arising from a [911 request for medical aid](#) or otherwise resulting in a patient contact, as defined in the clinical standard on [Definition of a Patient](#).

Intent:

To ensure that all incidents where units are dispatched for the care of a real or perceived patient are appropriately documented per TAC 103.4. The ideal outcome is for one ePCR per incident whenever a CAD number is generated for a medical request for service, whether or not medical care is delivered on scene. Key decisions will include what dispositions to use, as noted below, and who will complete the ePCR. Providers should err on completing an ePCR if they do not know who is responsible for the chart on an incident when multiple units are assigned. The goal of this standard is to ensure compliance with state law while respecting our clinicians' time.

Application:

If a unit is assigned to an incident and replaced by another unit from the same agency before arrival at the scene:

- a) No ePCR is required.

For all incidents:

- a) System agencies will ensure that providers can complete a patient care report within the time limits specified in this standard.
- b) One provider in each responding unit will be designated as responsible for completing the patient care report.
- c) Be truthful, accurate, objective, pertinent, legible, and complete with appropriate spelling, abbreviations, and grammar.
- d) Be completed within 24 hours of disposition of the incident.
- e) Remain confidential and be shared only with legally acceptable entities.
- f) Once an ePCR is completed, the original record will not be modified for any reason. Any changes required to correct a documentation error or to add clarification shall be recorded in an addendum.

For every incident where a unit is dispatched but is canceled before arrival on the scene, the following documentation requirements apply:

- a) Select a "canceled prior to arrival" disposition.
- b) Document any special circumstances in the narrative
- c) If multiple system organizations respond to an incident, at least one credentialed provider from each organization must complete a report that is accurate to their organization's activities.

For every incident where a unit is dispatched but does not make patient contact after arriving on scene, the following documentation requirements apply:

- a) Select a "no patient contact" disposition.

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- b) If a patient could not be located, document any efforts made to locate the patient in the narrative.
- c) If multiple system organizations respond to an incident, at least one credentialed provider from each organization must complete a report that is accurate to their organization's activities.

If the definition of a patient is met on an encounter and a patient-provider relationship is established, all care must be documented. A separate patient care report must be completed for each patient on a scene. For every patient contact, the following documentation requirements apply:

- a) Reflect the patient's chief complaint and a complete history or sequence of events that led to their current request or need for care.
- b) Contain a detailed assessment of the nature of the patient's complaints and the rationale for that assessment.
- c) Document a complete physical assessment, a [complete set of initial vital signs](#), and all other pertinent findings.
- d) Document anything specific to the patient's condition (RAAS for behavioral patients, loss of consciousness for head injury patients, blood thinner use in head strike patients, LVO assessment in patients with arm weakness, etc.).
- e) Document ongoing monitoring of abnormal findings.
- f) Document any changes in the patient's condition.
- g) Summarize all assessments and interventions in chronological order and the results of the interventions with appropriate detail so that the reader may fully understand and recreate the events.
- h) For medication administrations, include the name, dose administered, route, administration time, indication, and patient response.
- i) For patients with extremity injury, note neurovascular status before and after immobilization.
- j) For IV administration, document the catheter size, site, any failed attempts, type of fluid, and amount of fluid administered.
- k) Include a lead II strip for all patients placed on the cardiac monitor. All 12-leads should also be included. Any significant rhythm changes should be documented. For cardiac arrests, the initial strip, ending strip, pre and post-defibrillation, pacing attempts, etc. should be attached. Or, electronically captured, uploaded and combined with the ePCR record.
- l) Document any requested orders, whether approved or denied, with physician's name.
- m) Document any waste of narcotics including the quantity wasted, where wasted, and must have the name of the person who witnessed the waste in appropriate software.
- n) Include an explanation for why an indicated and appropriate assessment, intervention, or action prescribed by the Clinical Operating Guidelines did **NOT** occur.
- o) If multiple System Organizations are on the scene, at least one System Provider/Responder making patient contact from each response organization is responsible for documenting ALL interactions, assessments and treatments their response organization provided to the patient on a ePCR for their organization. All system-credentialed providers involved in patient care must be documented in the ePCR(s). All system-credentialed providers are responsible for the documentation of

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patient care and pertinent elements of the history and physical examination during their phase of care.

- p) If any clinical interventions are performed by a system credentialed provider prior to the arrival of the transporting asset, completed [patient notes](#) (or substantially similar) are to be provided to the transporting asset. These clinical notes are to be transported with the patient to the receiving facility. This should include, at a minimum: patient name, treatments rendered, and the times that such treatments were rendered.